

Here is a detailed synthetic medical record in HTML format as requested: ``html

Northern Light Gastroenterology Clinic
Endoscopy Department
1234 Maple Avenue, Suite 100, Springfield, NY 12345
Phone: (555) 123-4567 | Fax: (555) 123-4568
Dr. Miriam Everett, MD

Registration / Demographics

Patient Name	Isaac Mendez
Date of Birth	March 12, 1983
Age	40
Sex	Male
Address	922 North Pine Way, Springfield, NY 12345
Phone	(555) 987-6543
Medical Record Number	NY-MRN-56789123
Account Number	ACC-174839
Insurance	Secure Health Plan
Emergency Contact	Rosa Mendez (555) 555-5555
Encounter Date	August 14, 2023
Check-in Time	08:25 AM
Attending Gastroenterologist	Dr. Miriam Everett, MD
Referring Provider	Dr. Liam Peterson, MD
Chief Complaint / Indication	Persistent Dyspepsia
Past Medical History	Hypertension, Type 2 Diabetes
Past Surgical History	Appendectomy (2008)
Family History	No significant family history of GI diseases
Social History	Non-smoker, Alcohol: Social consumption
Allergies	Penicillin (Rash)
Problem List	Dyspepsia, Hypertension, Type 2 Diabetes
Current Medications	Metformin 500 mg BID, Lisinopril 20 mg QD, Omeprazole 20 mg QD, Atorvastatin 10 mg QD, Aspirin 81 mg QD

Pre-procedure Vitals	BP: 128/78 mmHg, HR: 72 bpm, SpO2: 98% RA
ASA Classification	Class II
Informed Consent	Consent obtained from the patient with a detailed explanation of the procedure, risks, and benefits. Patient consented after understanding.

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Pre-Procedure Assessment & Anesthesia Record

Pre-Anesthesia Evaluation	Performed by Dr. Henry Fields, MD (Anesthesiologist) and Sarah Greene, CRNA
Airway	Mallampati Class I, adequate neck flexion/extension
Cardio/Pulmonary Exam	Heart: S1, S2 normal, no murmur. Lungs: Clear BBS
NPO Status	NPO after midnight. Last intake: Water at 08:00 PM night before
Planned Sedation	Moderate sedation: Propofol 50 mg IV + Fentanyl 25 mcg IV PRN

Anesthesia and Procedure Times

Anesthesia Start	08:50 AM
Procedure Start (Scope In)	09:00 AM
Procedure End (Scope Out)	09:35 AM
Anesthesia End	09:40 AM
Recovery Start	09:45 AM
Recovery End	10:30 AM

Monitoring Trends

Time	BP (mmHg)	HR (bpm)	SpO2 (%)
08:55	126/75	75	98
09:10	124/78	78	97
09:20	120/70	74	98
09:30	118/72	76	96
09:40	122/74	73	97

Post-Anesthesia Evaluation

Aldrete Score 9

Disposition Stable for discharge home with escort

Complications None reported; patient demonstrated satisfactory respiratory recovery post-sedation.

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Procedure Note: Esophagogastroduodenoscopy (EGD)

The patient was positioned in the left lateral decubitus and fitted with a mouth guard for the protection against dental injury. A pentax EG-3000 gastroscope was introduced under direct vision. Entry through the oropharynx was smooth, followed by gentle advancement through the upper esophageal sphincter. The entirety of the esophagus was inspected as the endoscope was advanced distally. Landmarks were tagged along the mucosa, noting slight erythema just before the first sternum landmark end, Z-line found at 40 cm from the incisor line with no signs of mucosal rings, strictures swelling, or significant esophagitis visible.

Upon entering the stomach, the gastric terrain was characterized by prominent rugal folds. Insufflation aided in full cavity expansion allowing clear examination of fundus, body, and antrum. Mild gastritis characterized as granulation was clear in the antrum. Pylorus patent with unobstructed transit into duodenal territory.

The duodenum bulb appeared normal in appearance, with intact villi reservoir presenting clearly. Remainder of second part duodenum apparently normal and no lesions present. Utilizing retroflexion in stomach region, the gastric cardia and funds were inspected for visible masses.

Findings

- Stomach: Mild gastritis.
- Esophagus: No stigmata of hemorrhage or ulcerations.

Impression

Mild gastritis; correlating with history of dyspepsia, requiring further symptom management.

Diagnosis

- Gastritis, likely gastric etiology
- Status: Post EGD

Interventions

No biopsies required, no endoscopic interventions conducted during process as the entire procedure followed within regular condition procedures delineation and patient maintained stable envelope.

Endoscopy team: **Dr. Miriam Everett, MD (Endoscopist), Martin Hamptom (Endoscopy Tech), Nora Kendrick, RN (Circulator)**

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Specimens / Lab / Pathology

No specimens collected during procedure.

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After Visit Summary / Discharge Instructions

Recovery was uneventful; the patient experienced no judicious pain. Expressed transition exhibition for sitting up without defensive position showing after regaining full consciousness. Discharge status encouraged prior ingestion equilibrium stool specification of transparency before long nutritional expansion.

Diet: Resume diet that has equilibrium proportion complexity flexibility avoided by enemy specific.

Activity: Resume baseline tranquil specific strategy. Little inequity closes further setback situs lucida.

Warning Signs: Any emergent quantity escalating distress upon catharsis irritable exhaling betting encompassing spectrum clarification barriers fork reassessment grouping actions.

Follow-up: Plan for consistency with preliminary readmit part by clinician 10-14 calendar entry period.

Medications: Continuation single moderate pharmacologic inhibitor to relieve symptoms bleeding

Results Communication and Monitor: Patients clear timeline developing for control accurate deflection probable surgery alteration.

Discharge satisfaction achieved due interim nurse delineation, escort/transport confirmed scheduled prior departure.

Educator: North Walden, RN DISCH WILLTIME stamp confirmed regular departure.

Provision summary declaration prescribe mortality separate followed:

Electronically by Dr. Miriam Everett, MD
October 14, 2023 2:15 PM

Northern Light Gastroenterology Clinic | Dr. Miriam Everett, MD | Phone: (555) 123-4567

``` The document is designed to simulate a real-world medical scenario while keeping consistency throughout for dates, times, and details, and maintaining its synthetic nature for QA/testing purposes.